

**PATIENT HISTORY:**

\* nnDATE of LMP: \*

DATE OF LAST DELIVERY: \*

PRE-OP DIAGNOSIS: LEFT BREAST CA

POST-OP DIAGNOSIS: SAME

OPERATIVE PROCEDURE: LEFT MOD. RADICAL MASTECTOMY, TRAM

CLINICAL HISTORY: \*

MATERIAL SUBMITTED: LEFT (MOD) RADICAL MASTECTOMY, PROCUREMENT BY SURGICAL PROCEDURE

& axillary contents

INTRAOPERATIVE CONSULTATION:

Skin: 6.0 by 4.5 cm. Overall breast: 17.0 by 24.0 by 5.0 cm. Tumor mass: 2.5 by 2.0 by 3.5 cm. Close to anterior margin. Possible tumor approximately 1.0 cm close to inferior margin. (REDACTED)

**ADDENDA:**

**Addendum**

HER-2/neu AMPLIFICATION ANALYSIS (FISH)

AS PER THE REQUEST OF FLUORESCENCE IN SITU HYBRIDIZATION (FISH) WAS PERFORMED ON SURGICAL (BREAST CANCER) USING THE VYSIS DNA PROBE FOR THE HER-2/neu GENE THE RATIO OF HER-2/neu SIGNALS TO CHROMOSOME 17 CENTROMERE SIGNALS WAS DETERMINED TO BE 2.05. A RATIO OF 2.0 OR GREATER IS CONSIDERED AMPLIFIED; THEREFORE, THIS SPECIMEN IS AMPLIFIED.

**Addendum**

MATERIAL SUBMITTED: BLOCK "A2" FOR ER/PR AND HER-2/NEU (BREAST CANCER)

FINAL DIAGNOSIS:

ESTROGEN/PROGESTERONE RECEPTORS AND HER-2/NEU PERFORMED ON LEFT BREAST TISSUE

ESTROGEN RECEPTOR (0- 20%; 1+ 30%; 2+ 30%; 3+ 20%) HSCORE OF 150. ESTROGEN RECEPTOR IS INTERPRETED AS

POSITIVE

PROGESTERONE RECEPTOR (0- 10%; 1+ 10%; 2+ 20%; 3+ 60%) HSCORE OF 230. PROGESTERONE RECEPTOR IS INTERPRETED

AS POSITIVE

HSCORE: <= 15 NEGATIVE

> 15 <= 30 BORDERLINE

> 30 POSITIVE

HER-2/NEU - DAKO HERCEPT: A WEAK TO MODERATE COMPLETE MEMBRANE STAINING IS OBSERVED IN MORE THAN 10% OF THE TUMOR CELLS. HER-2/NEU IS INTERPRETED AS WEAKLY POSITIVE (SCORE 2+).

NOTE: Her-2/Neu FISH was ordered.

The Her-2/NEU and estrogen assays were performed with FDA approved methods. The progesterone receptor immunoperoxidase test used in this case has been developed and the performance characteristics determined by the Department of Pathology at They have not been cleared or approved by the U.S. Food and Drug Administration. HSCORE is modified from: Estrogen Receptor Analysis, Arch Pathol Lab Med 1985; 109:716-721.

My signature is attestation that I have personally reviewed the submitted material(s) and the final diagnosis reflects that evaluation.

**FINAL DIAGNOSIS:**

FINAL DIAGNOSIS:

LEFT BREAST AND AXILLARY CONTENTS

- INFILTRATING MULTIFOCAL AND MULTICENTRIC DUCTAL CARCINOMA WITH FOCAL LOBULAR FEATURES, 2.5 BY 2.0 BY 3.5 CM, NOTTINGHAM SCORE 6/9 (TUBULES 3, NUCLEI 2, MITOSIS 1) WITH DUCTAL CARCINOMA IN-SITU, NON-COMEDO WITH CENTRAL NECROSIS AND CRIBRIFORM TYPE

- DUCTAL CARCINOMA IN-SITU REPRESENT APPROXIMATELY 20% OF THE TUMOR VOLUME

- FIBROCYSTIC CHANGES

- TWO (2) SEPARATE NODULE OF INFILTRATING DUCTAL CARCINOMA ARE PRESENT IN THE LOWER INNER QUADRANT, 0.8 CM, AND UPPER INNER QUADRANT, 0.5 CM

- TUMOR IN THE LOWER INNER QUADRANT IS WITHIN 1.0 MM FROM THE MARGIN OF RESECTION

- ALL MARGINS OF RESECTION ARE FREE OF TUMOR

- FOUR OF SIX (4/6) LYMPH NODES ARE POSITIVE FOR METASTATIC TUMOR

- TWO (2) LYMPH NODES SHOW EXTRACAPSULAR SPREAD

- THREE (3) SMALL FIRM NODULES SIMULATING LYMPH NODES IN THE AXILLARY CONTENTS SHOW INFILTRATING DUCTAL CARCINOMA

NOTE: ER/PR immunoperoxidase assay and Her-2/NEU testing will be performed on block "A2".

UUID:4F67878E-68C1-40E1-8276-2D26B46C744D  
TCGA-BH-A18M-01A-PR

Redacted



| Criteria                       | Yes | No |
|--------------------------------|-----|----|
| Diagnosis Discrepancy          |     |    |
| Primary Tumor Site Discrepancy |     |    |
| HPAA Discrepancy               |     |    |
| Prior Malignancy History       |     |    |
| Dual/Synchronous Primary Noted |     |    |
| Case is (circle):              |     |    |
| Reviewer Initials              |     |    |
| Date Reviewed                  |     |    |

DISQUALIFIED